

D7 — Cardiology Consult

Dr. A. Kim, Cardiology | Cardiology | 02/20/2026 09:00

Patient: Pemberton, Harold J.**MRN:** 847-29-3156**DOB:** 04/12/1951**Allergies:** Sulfa drugs (rash)

CARDIOLOGY CONSULT NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156
Date: 02/20/2026 09:00 | Provider: Dr. A. Kim, Cardiology
Reason for Consult: Acute on chronic heart failure exacerbation

- ECHOCARDIOGRAM (02/20/2026):
- EF 30% (likely chronic, consistent with prior studies)
 - Moderate mitral regurgitation
 - Dilated LV
 - Diastolic dysfunction grade II
 - Wall motion abnormalities in LAD territory (chronic)
 - No pericardial effusion

- ASSESSMENT:
- Acute on chronic systolic heart failure exacerbation, likely precipitated by:
1. Volume resuscitation in ED (1L NS bolus in a patient with EF 30%)
 2. Systemic corticosteroids for COPD exacerbation (sodium/water retention)
 3. Concurrent COPD exacerbation creating additional cardiopulmonary stress

BNP trajectory: 480 (admission, likely near-baseline) → 2,400 (Day 2) confirms acute decompensation superimposed on chronic CHF.

- RECOMMENDATIONS:
1. IV furosemide 80 mg BID – aggressive diuresis. Target net negative 1-2L/day.
 2. Continue carvedilol 25 mg BID (essential for HFrEF mortality benefit)
 3. Continue lisinopril 20 mg PO daily
 4. Continue spironolactone 25 mg PO daily
 5. Continue amlodipine 5 mg PO daily
 6. Transition IV steroids to PO prednisone 40 mg daily and begin taper to minimize further fluid retention
 7. Apixaban: continue home dose 5 mg BID
 8. Daily weights, strict I/Os, fluid restriction 1.5L/day
 9. Repeat BNP in 48-72 hours to assess response to diuresis

Will follow daily.

Electronically signed by: Dr. A. Kim, MD

Date: 02/20/2026 10:15

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